

WHO Guidelines: Diabetes Mellitus

1. Diagnostic Criteria for Diabetes Mellitus

Diagnosis of Diabetes Mellitus is established if any of the following criteria are met:

1. Fasting Plasma Glucose (FPG) ≥ 126 mg/dL (7.0 mmol/L) after an 8-hour fast.
2. 2-hour Post-Prandial Glucose ≥ 200 mg/dL (11.1 mmol/L) during a 75g OGTT.
3. Glycated Hemoglobin (HbA1c) $\geq 6.5\%$ (48 mmol/mol).
4. Random Plasma Glucose ≥ 200 mg/dL (11.1 mmol/L) in patients with classic symptoms of hyperglycemia (polyuria, polydipsia, unexplained weight loss).

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2. Prediabetes Categories

Prediabetes indicates high risk for progression to Type 2 Diabetes:

1. Impaired Fasting Glucose (IFG): Fasting glucose 100-125 mg/dL (5.6-6.9 mmol/L).
2. Impaired Glucose Tolerance (IGT): 2-hour OGTT glucose 140-199 mg/dL (7.8-11.0 mmol/L).
3. HbA1c range: 5.7% to 6.4% (39-47 mmol/mol).

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3. Pharmacological Management

First-line therapy for Type 2 Diabetes is Metformin combined with lifestyle intervention.

- SGLT2 Inhibitors (Empagliflozin, Dapagliflozin) are recommended for patients with established ASCVD, Heart Failure, or Chronic Kidney Disease.
- GLP-1 Receptor Agonists (Semaglutide, Liraglutide) offer strong glycemic efficacy and weight loss benefits.
- Insulin therapy is indicated when HbA1c > 10% or symptoms of catabolism are present.